

MRI of the Left Ankle

09/12/2023

Clinical Information: Left ankle pain.

Findings:

A vitamin E capsule was placed over the medial ankle, where the patient feels pain. Subjacent to the marker, there is a semicircular subchondral lesion involving the medial talar dome with associated mild marrow edema, measuring 8 x 8 x 2 mm (AP/T/CC), consistent with osteochondritis dissecans/osteochondral lesion.

There is a small tibiotalar and posterior subtalar joint effusion. The rest of the imaged osseous structures demonstrate no evidence of occult fracture or bone contusion. An os trigonum is noted demonstrating subtle marrow signal changes, for which minimal marrow edema is not excluded.

There is a small amount of fluid surrounding the flexor hallucis longus tendon, consistent with mild tenosynovitis. The rest of the extensor and flexor tendons, as well as the Achilles and peroneal tendons are normal. An incidental peroneus quartus muscle is noted. The anterior talofibular, calcaneofibular, and posterior talofibular ligaments are normal. The deltoid ligament is intact. The plantar fascia is unremarkable.

Impression:

1. 8 mm osteochondral lesion/osteochondritis dissecans with mild marrow edema involving the medial talar dome. This lies subjacent to the site of medial ankle pain indicated by the patient.
2. Os trigonum with subtle marrow signal changes. Minimal marrow edema/possible mild os trigonum syndrome is not entirely excluded.
3. Small tibiotalar and posterior subtalar joint effusion.
4. Mild flexor hallucis longus tenosynovitis.
5. Incidental peroneus quartus muscle (anatomic variant).